

PACKAGE INSERT

CROTAMIXONE CREAM

Composition:	
Hydrocortisone Acetate	0.25% w/w
Crotamiton	10% w/w

Product Description: A smooth, white and soft cream.

Pharmacodynamics:
Crotamiton has scabicial and anti-pruritic actions. The mechanisms of these actions are not known. Absorption of crotamiton in man from topical application is not determined.
Topical corticosteroids share anti-inflammatory, anti-pruritic and vasoconstrictive actions. The mechanisms of anti-inflammatory activity of topical corticosteroid is unclear. However, there is some evidence to suggest a recognizable correlation exists between vasoconstrictor potency and therapeutic efficacy in man. The extent of percutaneous absorption of topical corticosteroids is determined by many factors including the vehicle, the integrity of the epidermal barrier, and the use of occlusive dressing.

Pharmacokinetics:
Topical corticosteroids can be absorbed from normal intact skin. Inflammation and/or other disease processes in the skin increase percutaneous absorption of topical corticosteroids.
Once absorbed through the skin, topical corticosteroids are handled through pharmacokinetic pathways similar to systemically administered corticosteroids. Corticosteroids are metabolized primarily in the liver and are then excreted by the kidneys. Some of the topical corticosteroids and their metabolites are also excreted into the bile.

Indication:
It is indicated for the treatment of scabies and symptomatic treatment of pruritic skin. Eczema and dermatitis of all types including atopic eczema, photodermatitis, otitis externa, primary irritation and allergic dermatitis, intertrigo, prurigo, nodularis, seborrheic dermatitis and insect bites reactions.

Recommended Dosage:
A thin film of cream is applied to the effected skin areas two to three times a day. Occlusive dressing should not be used. Treatment should be limited to 10-14 days or up to 7 days if applied to the face.

Route of Administration: Topical

Contraindications:
It is contraindicated in patients who are hypersensitive to hydrocortisone and crotamiton or to any components of the preparation. It should not be used for bacterial, viral or fungal infections of the skin and acute exudative dermatoses. It should not be applied to ulcerated areas.

Warnings and Precautions:
If severe irritation or sensitization develops, treatment with this product should be discontinued. It should not be applied in the eye or mouth because it may cause irritation.
It should not be allowed to come into contact with the conjunctiva and membranes. Systemic absorption of topical corticosteroids has produced reversible HPA axis suppression, manifestations of Cushing's syndrome, hyperglycemia and glucosuria in some patients.
Therefore, patients receiving a large dose of a potent topical steroid applied to a large surface area or under an occlusive dressing should be evaluated periodically for evidence of HPA axis suppression by using the urinary free cortisol and ACTH stimulation tests. Long term continuous topical therapy should be avoided since this can lead to adrenal suppression even without occlusion. If HPA axis suppression is noted, an attempt should be made to withdraw the drug, to reduce the frequency of application, or to substitute a less potent steroid. Recovery of HPA axis function is generally prompt and complete upon discontinuation of the drug.
Potent corticosteroids have been shown to be teratogenic after dermal application in animal studies. Therefore, topical corticosteroid should be used during pregnancy only if the potential benefit justifies the potential risk to the fetus. Drugs of this class should not be used extensively on pregnant patients, in large amounts, or for prolonged period of time.

Paediatric patients may demonstrate greater susceptibility to topical corticosteroid-induced HPA axis suppression, Cushing's syndrome and intracranial hypertension have been reported in children receiving topical corticosteroids. Manifestations of adrenal suppression in children include linear growth retardation, delayed weight gain, low plasma cortisol levels and absence of response to ACTH stimulation. Manifestations of intracranial hypertension include bulging fontanelles, headaches and bilateral papilledema.
Administration of topical corticosteroids to children should be limited to the least amount compatible with an effective therapeutic regimen. Chronic corticosteroid therapy may interfere with the growth and development of children. It should be used with caution in infants and for not more than 7 days.

Interactions with Other Medicaments:
No known drug interaction.

Pregnancy and Lactation:
Topical corticosteroids and crotamiton should be used during pregnancy only if the potential benefit justifies the potential risk to the fetus. Drugs of this class should not be used extensively on pregnant patients, in large amounts, or for prolonged periods of time.
It is not known whether topical administration of corticosteroids could results in sufficient systemic absorption to produce detectable quantities in breast milk. Nevertheless, caution should be exercised when topical corticosteroids are prescribed for a nursing woman. Nursing mothers should, in all events, avoid applying the preparation in the area of the nipples.

Side Effects:
It occasionally causes transient erythema and a sensation of warmth. Allergic sensitivity or primary irritation reactions may occur in some patients. Other side effects such as dryness, folliculitis, hypertrichosis, acneiform eruptions, hypopigmentation, perioral dermatitis, allergic contact dermatitis, maceration of the skin, secondary infection, skin atrophy, stria and miliaria have been reported under occlusive dressings.
Systemic absorption of topical corticosteroids has produced reversible hypothalamic-pituitary-adrenal (HPA) axis suppression, manifestations of Cushing's syndrome, hyperglycemia and glucosuria in some patients.

Symptoms and Treatment of Overdose:
Topically applied corticosteroids can be absorbed in sufficient amounts to produce systemic effect. (See Warnings and Precautions). There is no specific information on the effect of overtreatment with repeated topical applications in human.

Storage Conditions: Store at or below 30°C.

Pack Size: A tube of 15g and 30g.
Pack Size (export only): A jar of 400g.

Shelf-life: 2 years.

FURTHER INFORMATION CONCERNING THIS DRUG CAN BE OBTAINED FROM YOUR FAMILY PHYSICIAN / LOCAL GENERAL PRACTITIONER / PHARMACIST.

Manufacturer & Product Registration Holder:
SUNWARD PHARMACEUTICAL SDN. BHD.
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EL205A-R5
Revision date: 23/10/2025